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Health and information records management

health records and information technology (Kenya Medical Training College)

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A C K N O W L E D G M E N T

It is not very easy to thank each and everybody who contributed to the successful production of this manual. But I wish to take this opportunity to thank a few outstanding individuals.

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Thanks also go to…….who managed to type this manual and to Health Learning Material Project for Printing this manual.

Last but not least, more thanks go to the rest of my family who have been patient with me during this period of writing the manual away from home

2. Give a historical background of health records discipline.

A health record is any written information about a patient in a professional relationship with a doctor.

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The history of health records runs parallel with the history of medicine. Records are as necessary for the practice of medicine as medications are for effective treatment, and they seem to have been made from earliest antiquity.

The first real physician of health records in Egypt is Imhotep. He lived in the pyramid age (about 3,000 – 2,500 BC) and was grand vizier, century before Christ. Imhotep has been credited with the authorship of the Edwin Smith Papyrus. This papyrus is one of the most valuable ancient medical documents that has come down to us and appears to have been copied about 1600 BC form an earlier original. This papyrus is a roll over 15 feet long by about 13 inches wide and is made up of 12 sheets of the usual size. It is written on both sides and consists of 48 cases of clinical surgery.

In Greece Hippocrates, known as the "father of medicine" was born about 460 BC. He was the first to cast out superstition and to practice medicine on scientific principles. He was the author of the Hippocratic Oath, which is pledged by physicians and which states in part: "whatsoever in my practice or not in may practice. I shall see or hear amid the lives of men which ought not to be noised abroad – as to this I will keep silence, holding such things unfitted to be spoken." Thus originated the privacy of all information given to the physician by patients, eventually the health records per see was, also considered as a privileged communication. Hippocrates kept detailed case reports of his patients.

In the 18th century Benjamin franklin was one of the first leaders in the movement to establish the first incorporated hospital within the United States. This institution now known as Pennsylvania in 1751. Franklin served as secretary of the hospital, and many of its earliest records are in his handwriting.

In the 19th century in 1821 the famous Masachussets general ho

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spiral, Boston opened. It has the distinction of having a complete file of clinical records, with all cases catalogued; dating from the day it opened.

In the 20th century, it was not until the beginning of this era that medical records received serious consideration by other types of hospitals and especially by hospitals and medical association.

In 1902 the American Hospital Associations discussed Health Records for the first time at a convention.

In 1967, the department of health records was started in Kenya, Kenyatta national hospital, Nairobi.

In 1978, the Health Records and information technician programme was started at the Medical Training College, Nairobi.

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CHAPTER TWO

THE UNIT HEALTH RECORDS SYSTEM

Introduction

This is a health records system where one patient is given one number for the rest of his stay or attendance in one hospital.

Learning Objectives

Students should be able to:-

1. Define the unit system
2. Describe the health unit system.
3. Initiate the unit record
4. Prevent duplication of the unit number
5. Describe the contents of the unit health record.

The unit system is a health records system in which all health records notes relating to one patient are contained in one case folder – the patient is the unit.

Initiation of the Unit System

The change from the old system of numbering to the unit system presents no special difficulties. From any given day it is simply necessary to assign a unit number to every patient attending for the first time thereafter. A six-digit range of numbers is normally used ranging from 1 to 999,999. Some health records officers prefer to insert the thoughts at the beginning i.e. 000,001 but this arrangement is quite optional. When an old patient attends or is admitted at the first time his old notes are filed at the back of his new unit folder under the new unit number. Only the old records of patients who present themselves after the appointed day are brought forward in this way, the others remain in the old file under their original identification number. A tracer card should be replaced where the old record was indicating the new unit number.

Prevention of Duplication

Duplication in the use of unit numbers can be prevented:-

(a) Either by not giving the same number to two patients.
(b) Not giving the same patient more than one number.

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In addition to the above the patient can be asked whether he has attended the hospital previously during registration. Whether the patient says yes or no a cross-checking can be done through the patient master index to ascertain this.

The Unit Number Register

From the foregoing comments the importance of the unit system will be appreciated if the unit number register is maintained. In the unit number register the following information will be included:-

(a) Full names of patients
(b) Addresses
(c) Dates of birth
(d) Specialty to which they have been referred.

The contents of the unit health record

(a) The folder

When selecting a folder the following points should be noted:-

(i) Strength of Manila.
(ii) Method of fastening in documents
(iii) Clarity of numbering on the outside cover.
(iv) Cost of the folder

The folder should be made of tough manila with a gusset and single or double fold inside, through which are threaded metal or plastic prongs with which the loose sheets may be fastened into the folder.

(b) The front sheet or identification sheet

This is the area where all the patients' social details are transcribed form the pre-registration form. The inside front cover of the folder id sometimes used for this purpose. Space should be provided to record several changes of address and change of a doctor.

A section is often provided here to list the consultants whose clinics the patient has attended or to whose wards he has been admitted.

It is important that all these facts should be readily visible to the doctor who is in charge of the patient, and who may

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wish to remind himself at a glance of the personal facts about the patient without hunting through sheaves of documents. It is also important that the information should be complete and accurate so that the person writing a letter to the doctor will dress this correctly, or anyone writing to the patient will send the letter to the correct address.

(c) Clinical history sheet

Two types of clinical history are in common use. In one, each consultant has his individual history sheets on which he only will write. A patient currently attending three different clinics will have three separate history sheets although these are likely to be filed next to each other. An alternative way is the continuous history sheet on which each doctor writes as he sees the patient. This achieves a chronological statement about the patient which would seem to have many advantages. If the continuous history sheet is used, it is important that the different clinics that the patient has attended should be easily identifiable – perhaps by using different coloured rubber date\clinic stamps.

(d) Continuation sheet

Some patient's attendances may be so short that their entire hospital episode is contained on one history sheet. This is not very usually, and continuation sheets are similar to history sheets but without any special printed identification of consultant or clinic which enables them to be used by all consultants.

(e) Prescription chart

This chart makes formal provision for the doctor to prescribe drugs for in or out-patients. The form must always find its way back to the case folder form the pharmacy so that the doctor treating the patient can see what precious medication has been prescribed.

(f) Surgical operation sheet

Special forms are provided on which operations may be recorded. They normally contain sections in which the

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names of all surgeons and anesthetics taking part in the operation are to be recorded, and anesthetics taking part in the operation are to be recorded, and special remarks about drainage, blood loss, special recovery treatment etc.

(g) <u>Anaesthetic record</u>

An anaesthetic techniques becomes increasingly sophisticated and complicated, the need for a special form on which to record the relevant facts has frown. Most anaesthetic record forms contain a considerable amount of detail, filled in by the anaesthetist during the operation and immediate post-operative period.

(h) Temperature, pulse and respiration (TPR) Chart

There are two usual versions of this chart. One is the b.d twice daily chart for the recording of the routine observations. Urine and stool details are usually recorded at the foot of the T.P.R. chart. A separate chart is often used for 4 – hourly recordings for more seriously ill patients. This is frequently printed on coloured paper, or in coloured ink, to make it noticeable.

(i) <u>Report mount sheets</u>

Many reports, which are smaller than the size of most forms, have to be filed in the folder. They are usually mounted on a report mount sheet. These mount sheets vary in designs and method of gumming. There are two possibilities; all report forms may have a gummed edge and be stuck on to the mount sheet itself may have a series of gummed strips on to which all reports will be stuck. Several versions of the mount sheet with gummed strips are available. The most popular and most expensive has strips of self-adhesive gum protected with greases paper. The protective paper is peeled off and the edge of the report pressed on to the exposed gum. This procedure is so much agreeable than having to moisten the edge of a form usually done, unhygienic ally, with the tongue that its introduction is extremely popular with all members of staff who have to fasten reports into notes. The forms are filed one above the other, but with approximately one quarter

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inch or one-third inch of the lower edge left visible so that the date and type of report can immediately be seen. in this way, a dozen or more reports will be filed on one mount sheet.

(j) Consent forms

Written consent has to be obtained for all operative procedures (which are technically an assault on the patient.) for sterilization procedures, for post-mortem examination – to consent has to be obtained from patient, relative or parent (if the child is under 16). These forms will usually be fastened onto a mount sheet, often a separate one form those used for laboratory or other report forms.

(k) Report forms

All investigations – laboratory (hematology, geochemistry, bacteriology, and histology), cardiology, radiological result in some sort of report form that will have to be fastened into patients notes. These reports are becoming rapidly a very special problem, as they swell the bulk of the folders in rapidly increasing numbers. Some laboratories issue cumulative report forms each on which in turn replaces all the reports that have been issued previously but unless the previous reports are conscientiously removed there is no reduction in bulk. Computerized laboratory reports have not yet provided an easy answer to this problem as storage space within the folder.

(i) Clinical photographs

When a patient has an unusually interesting condition, a clinical photograph may be taken. In plastic surgery, where the patient may pass through several stages of treatment, a photograph may be kept centrally in the intervals to record progress. Photographs may be kept centrally in the medical photograph department, but they are frequently filed in the case folder, either mounted on card or in a special plastic holder with pockets for the storage of a series of smaller photographs or slides.

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(M) in-patient summary
After a patient has been discharged form hospital, it is usual for the registrar or consultant to dictate a summary (though this is frequently a substance ial document) of salient points of the patient's progress while an in-patient carried out operations or other treatment, and condition on discharge are always included. A copy of this summary is often used as the hospital's full report to the general practitioner. The summary usually forms the document on which diagnostic coding is carried out.

(n) **correspondence**
Copies or originals of all communications form general practitioner to hospital doctor, and form hospital to general practitioner must be returned. These will include copies of the discharge note, which is traditionally written by the junior hospital doctor on the day of the patient's discharge, to keep the doctor informed of the most important points about his patient's stay in hospital until he receives the full discharge summary.

These are some of the more important documents normally included in the case folder, but there will be literally hundreds more in sue in most hospitals, their number and use depending on the type of hospital students will become familiar with these forms as with the case notes.

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CHAPTER THREE

DIFFERENT TYPES OF HEALTH RECORDS, THEIR VALUES, USES AND RESPONSIBILITIES OF A HEALTH RECORDS TECHNICIAN.

INTRODUCTION

This chapter will discuss different types of health records, their value and uses and the responsibilities of a health record/information technician.

Learning objectives

Students should be able to:

1. Identify different types of health records
2. Explain the uses and value of health records in:-
   - (i) Treatment
   - (ii) Planning
   - (iii) Research
   - (iv) Teaching
   - (v) Administration
3. Explain the qualities and responsibilities of a health record/information technician.

TYPES OF HEALTH RECORDS

There are 3 different types of health records:-

1. Case records
2. Out-patient records
3. Diagnostic records

1. Case records

   These are records initiated for patients who get admitted into the wards or who continuously attend the various consultant clinics. The information that is contained in the case can be expressed verbally, graphically, diagrammatically or in a tabular form. This depends on the person who is taking in the information. The range of documents to be included in the case record also depends on the local requirements although certain documents will be common to all hospitals

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2. **out-patient records.**
   This includes all the cards that are used in the out-patient departments, for example casualty cares, antenatal cards, immunization cares, and any other card that may be used in the out-patient department.

3. **Diagnostic records**
   These include notes on radiography, pathology, electrocardiography and other investigations that are usually initiated by report forms; these forms are usually 6”x 4” in size and are available in all the consultation rooms both in the out-patient and in-patient departments in a health care facility. They could be different colors for quick identification.

Values and uses of health records

Health records have various uses:-

1. Treatment
   The health record is the first and foremost of value in the present and future treatment of the patient. The individual record is a reminder to the physician, surgeon, clinician, nurse, social workers or health technicians of what he or she has personally observed during the patient’s illness. It contains information of what has been observed by others. It provides or should provide the whole previous hospital record of the patient, so that every fact that may be important is permanently available for reference at any time. A compete record prevents duplication of work and effort and facilitates future care of the patient.

2. Planning
   Health statistics and information gathered from the health record will definitely be very useful in the planning of health services. It is very necessary that these statistics should be very accurate and disseminated promptly to the users:-. Inadequate records mean incomplete statistics – leading to

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improper planning of various activities in the health service.

3. Research
   Accurate recording of observations in the health record will lead to accurate information required for research. They should contain that basic information that is required of them to meet the purposes for which they are intended, one being research. The methods of filing and retrieving the records should be systematic and simple.

4. Teaching
   Health records can be used as an educational tool or instrument. When the quality of health records is high, the task of the student is simplified and conversely, when the quality is poor the student's task is made more difficult and his progress thereby impended.

5. Administration
   Complete health records mean increased and efficient service to the public, avoiding vexation, litigation and fair settlement of claims and capacity to answer enquiries about how the hospitals work.

   For the record to meet and be useful for the above purposes it should be designed for the purposes for which it is to be used. It is important for the record to meet the requirements of both the clinician and the records personnel. The record also should be complete. To be complete the record should be analyzed quantitatively by the health records officer and qualitatively by the clinician.

1. QUALITIES OF A HEALTH RECORDS TECHNICIAN
   Most of these qualities apply to most of the employees in a health care delivery service. A trained health records technician should have:-

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i. Integrity- quality of being honest morally upright.
ii. Adaptability- should be flexible to the circumstances in an environment
iii. Discretion- ability to use his own knowledge to make decisions
iv. Politeness- humility
v. Calmness – a state of not being emotional in an emergency situation.
vi. Neat appearance – smart and acceptable appearance in his work.
vii. Consistent efficiency – effective and exceptional performance in his work.
viii. Personal sense of vocation and service to the other health workers and thence the patient

2. <u>Duties and responsibilities of health records technician</u>

The health records technician should be able to:-

1. Manage and maintain health records and information services.

2. Manage and maintain health records and information systems
   (a) Initiate, collect, store and retrieve health records
   (b) Collect, tabulate, analyze, interpret and store health information.
   (c) Disseminate health information and provide feedback.
   (d) Establish good public relations
   (e) Plan, supervise, co-ordinate work in health records services

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(f) Maintain legal aspects and security of health records.
(g) Maintain health records equipment
(h) Maintain health patient appointments
(i) Classify code and index diseases.
(j) Schedule patients appointments.
(k) Edit records and provide quality Budget and control health records equipment, supplies and medical stationery.
(l) assurance
(m) Provide first aid.
(n) Receive, register, and admit and discharge patients
(o) Establish mechanism for patients' follow-up.

(p) Handle disaster and special records conduct/participate in basic operational research.
(q) Participate in teaching health records and information students and other health workers and community on health records.
(r) Design various health/clients forms
(s) Control client forms and finance

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CHAPTER FOUR

FUNCTIONS OF A HEALTH RECORDS AND INFORMATION DEPARTMENT

INTRODUCTION

This chapter explains briefly the functions of a health records department and the relationship with other departments in a health care capacity.

LEARNING OBJECTIVES

The students should be able to:-

1. Define a health records department
2. Enumerate the functions of a health records and information department.

Health records department is a department where health records/information is kept

The various functions of a health records department are:-

1) Reception
2) Registration
3) Admission
4) Discharge
5) Appointments
6) Filing
7) Tracing
8) Follow-up of patients
9) Clinic preparation
10) coding and indexing
11) Collection, tabulation, analysis and dissemination of information (statics).
12) Maintenance of health records equipment
13) Maintenance of the confidentiality of health records
14) Manage special health records
15) Design medical forms
16) Ensure quality assurance of health records

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1. Reception
   This is the reception of patients when they arrive in a health care facility. In this area the patient/client is greeted and welcome to the hospital.
2. Registration
   Registration is recording of identification details mostly social on documents needed for any attendance.
3. Admission
   The same identification details recorded during registration is used and the ward number added on the form.
4. Discharge
   This is a procedure carried when a patient is supposed to leave the hospital after treatment (in the ward)
5. Appointments
   Patients are asked to report to the various clinics on certain times and dates ready to be seen

6. Filing
   Is a way of arranging the documents in a prescribed order or in a systematic manner.
7. Tracing
   This is tracing/monitoring the movements of all the documents and their whereabouts.
8. Clinic preparation
   This is getting ready all the documents – 48 hours in advance before a patient attends a clinic
9. Follow-up of patients
   There are some patients who need follow-up after they have been discharged from hospital such as cancer cases.
10. Coding and indexing
    Diseases and operations and other procedures in medicine need to be coded and indexed using the international classification of diseases and the international classification of procedures in medicine (W.H.O.).

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11. Collection, tabulation, analysis, interpretation and dissemination of data
    Raw data collected from health records are put in table, analyzed, interpreted and forwarded to the users
12. Maintenance of health record
    All the equipment used in a health records department must be maintained by the health records and information technician.
13. Maintain confidentiality
    All the information in a health records document is confidential and should not be handled by unauthorized persons.
14. Manage special health records
    There are special health records that are initiated and handled differently from other records, namely:-
    (a) Psychiatric records
    (b) Tuberculosis records
    (c) Maternity records
    (d) Sexually transmitted disease records
15. Design medical forms
    All medical forms are supposed to be designed by the health records and information technician in consultation with the uses.
16. Ensure quality assurance of health records
    The quantity and quality of the record will reflect the type of health care being rendered to the patient/client.

Relationship between a health records department and other departments in a health facility.

The medical records officer in a health facility is expected to co-ordinate the day to day administrative functions with other departments in a hospital. Some of the departments in a health facility are listed below

1) Laboratory
2) x-Ray

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3) Radiography
4) Orthopaedic
5) Dental
6) Physiotherapy
7) Occupational therapy
8) Department of nursing
9) Department of oral health
10) Supplies
11) Accounts
12) Transport

All this co-ordination is done to enhance the treatment of the patient. Therefore the stands in the middle and all these activities revolved around the patient. No one department is more important than the other, they should work together towards the achievement of this common goal.

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CHAPTER FIVE
EFFECTIVE COMMUNICATION

This subject is seen fit to the discussed in the health records science though it is a management function because the health records and information officer needs it in his day to day works. It covers communication, process, skills and barriers to effective communication.

Learning objectives

Students should be able to:-

1. Define communication
2. Explain why we communicate
3. Describe the communication process
4. Describe methods and media of communication
5. Explain barriers to effective communication
6. Explain how to overcome these barriers

1. Communication

   Two definitions:-

   (a) The transmission and reception of messages
   (b) Any means by which a thought may be transferred.

2. Objectives of communication

   (a) To bet others to think or act the way we want them to think or act.
   (b) To inform others
   (c) To ask and answer questions
   (d) To listen to others (listening is one of the most important communication skills)

   Receiver

   Taste, smell and touch

   All convey messages to a receiver, for example, there is no sugar in our tea today. The bearing in that machine smells hot

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Silence

We sometimes forget that, we can also communicate through silence. This could show deep understanding of a speaker's words and a unwillingness to interrupt him, but it could also show our strong disapproval of what he said.

Barriers to effective communication

There are many barriers which make it difficult for the communicator to reach his goal. Some of the important barriers are:-

Age
Socio-economic
Language vocabulary
Attitudes-senders or receivers
Competition for attention

Age- the age difference between the sender and the receiver.
This age barrier almost always exists when two people of different age brackets are communicating with each other. A common language has to be found that will suit both the sender, who say happened to be a sophisticated middle-aged executive and the receiver and up-coming recent college graduate.

The economic gap between two different income groups is another barrier. For example, a two-car owner is not communicating when he tells come one who cannot even afford to take a taxi, that safety-belts should be installed in a car.

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Attitudes

Men of different political persuasions belonging to different schools of thought of political ideology or parties very seldom communicate effectively or influence each other.

Language

Barriers exist when the sender uses technical language to a group of laymen who have had no previous experience in the area.

Competition for attention

There is such competition for attention of the audience at any given time. To be an effective communicator, you must be able to be able to catch your audience's attention and hold it throughout the period of presentation. A good communicator should recognize and remove all communication barriers before starting to communicate.

How to overcome barriers

The sender must know his audience.

Background
Interests
Language

As a communicator you must remember that your audience is human not a machine or mere logs in a machine. Every audience is unique. Each responds to the sender's message according to his own drive, attitude, motivation, his education or lack of it, his social background and his interest.

As a communicator, it should be remembered that knowing your audience is not as simple as it sounds. One must understand human nature and human needs. Study your audience thoroughly and use language that your audience understands in order to minimize communication barriers, secondly, the

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message must be timely, meangful and applicable to the situation.

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CHAPTER SIX

RECEPTION, REGISTRATION AND INITIATION OF PATIENTS\CLIENTS HEALTH RECORDS

INTRODUCTION

This chapter explains reception registration and how the patients\clients' records are created.

LEARNING OBJECTIVES

The students should be able to:-

1. Define reception, registration and initiation of a health record.
2. Receive patients\clients.
3. Register patients\clients.
4. Initiate patients\clients health records.

1. RECEPTION OF PATIENTS\CLIENTS

Reception is the art of greeting and welcoming. The right person should be selected for this work, because he is the one who will interview the patient. He or she should have ease, assurance of manner and a pleasant appearance. He has to convey to the anxious patient that there is nothing to worry about; the hospital is smoothly run, created for his care. This is not a job for a neurotic or anxious person. It requires cool efficiency, the ability to convey confidants, patients and compassion – a very demanding combination of qualities.

2. REGISTRATION OF PATIENTS

Is the completion of a documentation of personal and health data before a patient is treated?

Registration falls into two procedures – that for out-patient consultative clinics the registration should be carried out before the patient attends the clinic. The environment should be conducive and the patients should be interviewed individually and in privacy. The patient is registered, a file opened and an appointment is given prior to the clinic day.

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Another important factor that should be noted is that the patient should only be given one unit number and the patient should be asked whether he has attended that hospital before. If the answer is no is when a new number should be given to prevent duplication of numbers. Whether the answer is yes or no, this should be cross-checked with the patient master index.

3. INITIATION OF PATIENTS RECORD

The identification details that are taken during the registration time I used to create the patient's file. The registration details are:-
Patient's full names, date of birth, hospital number given, address, occupation, marital status, religion, name of next of kin, address of next of kin.

Many hospitals have formed formal systems of pre-registration – this is sending the patient a simple form to be completed. This form will ask for all the identification details to be confirmed. Any form used should be as simple as possible to make it easy for the patient to complete it. This form, when returned will be used to create the patients'\clients' health record. The use of mechanical documentation ensures continuing use of the same date throughout the patient's stay in the hospital.
The master index cards should also be created during the time that the record is being created and filed immediately. This will help to answer enquires incase the patient happens to lose the attendance card.

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CHAPTER SEVEN

HEALTH RECORDS INDICES

INTRODUCTION

In this chapter the main health records indices are described, namely the master index, the diagnostic index, the operations index and the waiting list index.

LEARNING OBJECTIVES

The students should be able to:-

1. Define various health record induces.

2. Explain the importance of health record induces.

3. Describe the procedures used in creating health records indices.

4. Describe various equipment used in storage of health records indices.

5. Describe the techniques used in filling health records indices.

INDICES

There are various health records indices used and maintained in a health records department, namely:-

1. The patient master index.
2. The diagnostic index.
3. The operations index.
4. The waiting list index.

1. The Patient Master Index

Definition

The patient Master Index is the alphabetical key to the numerically filed case records.

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Importance

This is a very important index in the case where a patient has lost his attendance cards.

The following details must appear on any index card:-

Surname
First names
Date of birth
Sex
Hospital number
Address
Space left for change of address. A typical layout of the card which measures 3 *21\2 “or 5” *3” would be

3”

Name of Patient	Hospital No.
	D. O. B.
Address	Sex
Space for change of Address	

Figure 2 – PATIENT MASTER CARD

Creation of the master

The master index card is created when the patient is being registered.

Equipment used for filing the master index cards

Cards
Strip index
Elevator files
Cartwheel
Carousel type files
Guide cards

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Cards

Cards will be the basic record document in most master indexes. Generally a card must be of good quality for endurance.

Drawer filing

Cards may be filed “blind” in drawers – filed one behind the other in an upright position.

Strip index

A strip index as its name indicates consists of a series of thin wooden strips coated with paper on which the details required for the master index are filed on metal panels. The strips are filed on metal panels with a fold of metal down each edge which holds the strips in place.
The panels are hanged on a central revolving spindle so that they fan out like leaves on an open book. They may also be wall mounted. Third is recommended for the waiting list and not the master index.

Elevator files

In an elevator file the cards are usually laid in trays about a foot ling, and several of these trays are filed on a shelf in the machine. The machine looks like a large metal cabinet which may be up to five feet highland up to 10 feet wide, with an opening at the top. In this opening two or three of the shelves with their trays of cards are visible. The largest elevator file would probably hold 9000,000 index cards. The large files are electrically driven. If the card is wanted, the filing clerk presses a button indicating the shelves, the cards rotate until the correct one arrives at hand level. The clerk then looks in the appropriate tray for the card. Smaller elevator files are manually controlled.

The advantage is that there is speedy access to all cards while the clerk is comfortably sitting on a chair.

The disadvantage is that the machine is expensive and may not be afforded by some of the health facilities.

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Cartwheel type files

In this file the cards are attached to a large upright central wheel which is rotated until the required section comes to the top. This type of file is probably not suitable for the very large hospital.

Carousel type files

In this type the cards are housed in open metal boxes which are attached to a central upright spindle. The boxes can be swung out for use and then folded back towards the spindle when finished with. Because the boxes are attached right round the stem, several clerks can have access at once. The equipment is space dividing considering the number of card that can be housed.

The notes carrying a unique hospital number

There is always one or two departures form the unit system in hospitals which use this system. To maintain the confidentiality, some of the departments like psychiatry may keep detailed notes about their patients. But the run bridge report of 1965 and the walker report published in Scotland in 1967 advocates the inclusion of all the patients notes even if he attends any other specialty put together in one folder. We only need to mention this although this should not be the case.

Guide cards and back stops

All the equipment described above except the strip index will need some guide card or back stop. The function of the guide card is to sub-divide the file into reasonable sections; so that too much time is spent hunting through several hundred cards before finding the precise section of the file that is needed. Whatever the type of guide card chosen, clear, bold lettering will aid both filing and finding of cards. Backstops are the pieces of metal that stop the cards in a drawer that is not full from sliding down. Back stops should be firm enough to stay in place when a drawer is closed too roughly, but mobile enough to be readily adjustable as the cards have to be expanded backwards.

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Filling techniques used in filing master index cards

(a) Sorting

The most important step in filing is sorting – which means putting the cards into good before they are filed.

There are two methods of sorting most appropriate to a master index:-

(i) Use of sort-file – this is a series of flaps each bearing a letter of the alphabet – arranged on a solid base.

(ii) Pigeon holes – each lettered for the initial sorting of the index cards.

(b) Filing

This requires a lot of concentration. It is easy to file in a cord when the files in format and behind have been checked. This should be allocated to an experienced clerk.

(c) Checking

It is obvious that mistakes do occur when one is filing the cards and therefore constant checking of filed cards is required.

1. Reference to index

Once cards have been filed and checked they should only be removed when the address need to be changed. Cards should never be so packed as to prevent checking and filing.

The most important thing to note here is that the master index cards should be filed strictly alphabetically and only one member of staff should work on the cards at only one time to prevent cards form being misfiled.

2. The diagnostic index

The diagnostic index is to provide access for study purposes to the clinical data contained in the hospital health records. A health personnel wishing to carry out a study on any particular disease can obtain form the index the case folder numbers of those patients who have been admitted or have attended an out-patient department with a particular

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disease in which he is interested. The disease index will provide the number of the relevant case records and may also provide some minimal data about the patient's age, sex and outcome of the episode of treatment.

Equipment used to maintain the diagnostic index

(i) Vertical index card
(ii) visible edge card
(iii) Computers

Operations index

It will be possible for the health records and information officer to maintain an operations index besides the diagnostic index. Operative procedures need to be coded, just as do diagnoses. The code used is the classification of surgical operations.

Equipment used to maintain the operations index

The information is recorded on the dame media as the disease index. Identifying factors recorded on the operations index: case record number, sex, age, surgeon, outcome of episode and date of discharge.

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CHAPTER EIGHT

APPOINTMENT SYSTEMS

Learning objectives

The students should be able to:-

1. Define scheduling and follow-up.
2. Describe concept of scheduling and follow-up.
3. Explain the purpose of scheduling and follow-up
4. Describe the types of appointment systems.
5. Describe different sources of appointments.
6. Describe the equipment used to maintain appointments.
7. Explain how to prepare a clinic.

Appointment system is a systematic way of giving a specific date, time and venue of the clinic to an individual patient\client.

Concepts of scheduling

1. Patients should be distributed evenly to various clinics, depending on the number that the consultants will be able to see.
2. The staff manning the clinics should ensure that they are in time so that patients are not kept waiting for ling before they are seen.
3. Overloading of clinics should be avoided.
4. There should be a laid down policy on how to schedule urgent cases.

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5. Staff working in the appointment area should be familiar with the layout of the hospital so that they give proper directions to the patients.

Purpose of scheduling and follow-up

1. To reduce patients' waiting time.

2. To provide an even spread of work over the whole clinic session. An appointment provides an even spread of work among the medical staff running the clinic.

3. To allow the hospital to prepare for each and every patient in an advance so that delay for registration at the time of the clinic can be reduced.

4. To provide for special clinic arrangements, for example, recording of social history, weighing of patients, removal of plaster, pathological and radiological examinations.

5. To allow for distance and known transport difficulties. Allowance should be made by the health records staff for such factors as the distance a patient has to travel, train and arrival and departure times, rural transport difficulties, availability of local ambulance to the health care facility. All these can consistently ease the burden of attending the hospital.

6. To provide for teaching arrangements. A consultant may wish to select an interesting case to present to the medical students. This is simplified if an appointment system was well organized in a health institution.

Types of appointment systems

There are two types:-

Centralized

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Decentralized

Centralized

This means that all the appointments for the various clinics are made in one central place.

Advantages

1. The master index will be near the area for quick reference.

2. Control of staff, stationery and equipment by the health records\information officer is easy.

3. Each of the appointment clerks becomes familiar with the working systems of the various consultants.

4. All enquiries concerning appointments are referred to one place.

5. Urgent cases can be channeled quickly to the respective clinics.

6. When one clerk is sick or goes on leave other clerks continue with the work since they are all familiar with the work.

Decentralized

This is an appointment system which is carried on in different clinics.

Disadvantages

1. The master index is far from the appointment area.

2. Control of available resources is difficult.

3. When the clerk who mans the clinic becomes sick or is on leave the work comes to a standstill.

4. Enquiries concerning appointments are directed to different places.

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Advantages

1. The clerk dealing with the appointments becomes familiar with the patients and knows them by names.

2. The consultant in charge of the clinic will know the number of patients on his list without having to walk or ring the central area.

Some of the hospitals have the two systems combined. Return appointments being made in the various clinics and new bookings being done in the central area. It is the duty of health records\information officer to look at these systems critically and decide on which suits his hospital or health institution.

Sources of requests for appointments

1. Patients telephoning personally to make appointments.

2. Physicians on private practice wanting to book appointments for their patient through telephone or writing.

3. Letters from other health institutions given to patients to come and book their own appointments.

4. Patients already discharged from the hospital making return appointments

5. Patients referred from one clinic to another.

Equipment used to record appointments

The choice of the equipment used will depend on whether the appointment is being made centrally of decently.

(a) Loose leaf binders
(b) Visible edge sheets
(c) Diaries

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Loose leaf binders

Books vary in size and it is recommended that they be in loose leaf form to facilitate additions and removal of the sheets. The type to be used depends on the local circumstances.

Visible edge sheets

Sheets are cut out and by the use of kardex these sheets are fixed on the kardex and the information written on them can easily be seen when the kardex is open.

Diaries

Books that are bound can be used to write all the appointments for one consultant. The disadvantage is that as one keeps on cancelling the lists the diary becomes untidy. Another is that one cannot photocopy form the diary direct. He has to write out other lists form the diary.

Clinic preparation

Two days prior to the clinic all the clinic lists should be sent to the appropriate sections:- one to the clinic, one to the filing area, and another to the consultant in charge of the clinic area. All the records and documents should be pulled out, all the pathological reports inserted in the correct file and kept in the pigeon holes for the clinic receptionist to come and collect. The clinic receptionist should make sure that all the documents such as x-rays are ready for each and every patient attending the clinic. The clinics are prepared two days in advance to reduce the patients waiting time before he is seen by the physician. At the end of the clinic the receptionist should ensure that all the files are returned to the central library ready for filing.

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CHAPTER NINE

ACCIDENT AND EMERGENCY RECORDS PROCEDURES

INTRODUCTION

Accident and emergency records could not be any different form other records. This chapter will try to elaborate on the arrangements that are considered satisfactory as for as health records are concerned in this specialty.

LEARNING OBJECTIVES

Students should be able to:-

1. Identify the type of records to be kept

2. Describe how appointments are made in this department.

3. Describe how patients are disposed of form this department.

4. Describe the legal requirements to be observed in this department.

After a patient has been seen at the health centre he may be having some ailment which need immediate and urgent attention. Definitely the doctor will refer him to a hospital for immediate attention. He will find himself in an accident and emergency department. Also patients with any type of accident will be brought to this department directly.

1. Records to be kept
   (a) Firstly a register must be maintained at the accident and emergency department. The following information should be included In this

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register including name of patient, address, age, doctor referring him, time of arrival, brief description of injury, brief details of treatment, and then mode of disposal. The particulars of the person who has brought this patient to the accident and emergency department must be taken also. If it is a police officer his number must be recorded in the register. The register may be in loose leaf form or in bound volumes. Form the register statistics of attendance will be compiled.

(b) Clinical record

(i) Single card measuring 8”*5” or 6”*4” on which the identification details are recorded. For an R.T.A. (road traffic accident) patient the time and place of accident must be given and then space left for the clinician to write on.

(ii) Two part card

A card with carbonized part made from N.C.R. material. The card is made up in an envelope form so that x-ray reports and any other correspondence may be filed in it.

(iii) Thick paper envelops four and an eighth by five and three quarter inches (half the size of A4 paper.) The envelop will serve as a card and a pocket for reports.

2. Appointments

Most patients attending this department rarely come back for return appointment. For those patients that need return appointments this can be carried out in two ways:-

(i) Appointment register may be retained by the receptionist and the appointment written on the patient’s attendance card.

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(ii) Pre-printed appointment cards for each day and time may be given to the patient. A different colour of the card could be kept for each day of the week and when a patient arrives seeking an appointment a card is simply pulled out and given to him.

3. Disposal of patients

Disposal of patients attending this department could be in one of these categories:-

(a) Treated to finality and discharged.

(b) Treated initially and referred back to the nearest health centre.

(c) Referred to an out-patient consultant clinic.

(d) Referred to a consultant clinic in another hospital.

(e) Admitted to the wards for further treatment. Here full documentation for admission will be carried out before the patient goes to the ward.

(f) Transferred to another hospital for further treatment.

(g) In the case of a patient who has been brought dead (B.I.D.) the doctor just goes to certify the death and the body is conveyed straight to the mortuary.

4. Legal requirements

It is important to note that the same legal requirements that are applicable to other health records as far as retention is concerned still apply in accident and emergency records. They may be retained for a minimum period of six years after the last attendance. The records may be filed numerically or alphabetically depending on the number of records crated annually. At the beginning of the year, 1st

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January, a new file is started. The records should be kept in lick because most of these records are usually required in court and as much details as possible should be recorded. Most of the patients who have been involved in Road Traffic Accident may need some claims in future. Therefore statistics for these patients may be sent to the finance department so as to issue the necessary claims.

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CHAPTER TEN
WAITING LIST FOR IN-PATIENT AND DAY CASE TREATMENT

INTRODUCTION

This chapter will discuss what a waiting list is, types of waiting lists, advantages and disadvantages of each and how to maintain a waiting list.

LEARNING OBJECTIVES

Students should be able to:-

1. Define a waiting list
2. Describe the functions of a waiting list
3. Explain the types of waiting lists
4. Describe how to maintain a waiting list
5. Explain the method of filling to be used.
6. Describe procedures used in admitting patients.

A waiting list is an index of all patients waiting for admission to hospital or waiting for treatment on a day case bases.

1. Functions of a waiting list.
   The waiting list must be so organized that enquiries can be answered from individual patients wanting to know when they are supposed to be admitted in the health institution. Individual consultants who would like to know how many patients are on their waiting list should immediately be furnished with such information. The main function of the waiting list is to be able to make full use of the available beds in the health institution.

2. Types of the waiting list
   The waiting list may be maintained in various ways:-
   (i) Centralized waiting list

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This waiting list is held in one office and contains the names of all patients awaiting admission under all the consultants in the hospitals.

(ii) Decentralized waiting list

The decentralized waiting list will be maintained in several places, possibly by each consultant's secretary, or in individual wards or individual departments.

Advantages of the centralized waiting list.

1. Gives a fair representation of all the demands being made on the in-patient facilities.
2. All enquiries are referred to one place
3. Staff dealing with the waiting list develop skills in dealing with enquiries and in……..of the waiting list.
4. Updating ……change of address death are easily carried out.
5. Checking admissions and discharges from the daily returns is easier.
6. When one staff falls sick or goes on leave another staff can carry on with the work
7. Training of the staff becomes easier

Disadvantages centralized waiting list of

1. Consultants need to walk to the central office to select their patients from the waiting list.
2. The lists become so big so that some patients may be left out of the waiting list.

Advantages of decentralized waiting list

1. Consultants need not go to the central office to select their patients from the waiting list
2. The secretaries get familiar with the patients and can call them by name.

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Disadvantages of decentralized list

1. Several staff will be deployed in the maintenance of the waiting list in different areas
2. It will be very expensive since each department will need to use its own equipment.
3. A clerk will have to walk to the central records department to check for admissions and discharges from the daily returns.

3. Creation of the waiting list record
   Most of the waiting list records are initiated at the out-patient clinics. Some other patients may come from another hospital and be put in the waiting list for another different hospital, where there are more beds or facilities. There are four ways in which information can be conveyed to the waiting list:-

   (a) Card-The card is created for every patient who is to be put on the waiting list. This card will be filed and form part of the waiting list.

   (b) The nurse or doctor may send a list of patients to be included in the waiting list to the records department for action.

   (c) Letter- a consultant in one hospital may wish to include his patient's name in his waiting list, in another hospital for his name to be included in that waiting list. In this letter he will include diagnosis, and priority for admission. A case folder will then be created for this patient. The information to be included in this record are: the patient's name, address, title, telephone number, holiday dates, diagnosis, operation to be

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carried out, duration stay in the hospital, name of the surgeon, or consultant.

4. Method of filling
   A waiting list contains dozen names and needs two files:-
   1. Alphabetical index of the names of all patients on the waiting list. These cards should not be removed until the patient is admitted.

   2. Consultants list will make provision for date, time, and ward. These lists will have priorities indicated – routine, soon, urgent.

These two files should be able to answer an enquiry from a patient, a consultant.

5. Filling equipment

The type of equipment to be used will depend on the size of the list.

(a) Visible edge card filed in trays
When this is used to maintain a waiting list it may be in the form of manila flaps, with a rigid bar at the top and a transparent plastic pocket about half an inch deep at the lower edge of each flap. A card is inserted into this pocket and the flap is held in a metal tray in such a way that all the information in the plastic pockets is visible. Date put on the list, diagnosis, operation and admission priority is put to allow the consultant to select his patients.

(b) Strip index
A strip is limited in space so that no space is left for change of address. It can only be used if the list is not too long. It has already been mentioned as one of the equipment used in the maintenance of the patient master index card.

(c) Diaries

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Each consultant could have a diary for his patients put on the waiting list according to dates. Its danger is that a patient can easily be missed or overlooked on a date which has passed.

(d) Colour coding
Colour coding could be used to indicate soon, urgent and routine case

6. Procedures for admitting patients from list
   1. The patient is selected from the waiting list by the consultant,

   2. The clerk writes to the patient or telephones him inviting the patient to the hospital.
   3. The records are got out from the files and sent to the documentation office.

   4. The waiting list card is sent to the admission office so that the patient is expected on the day he comes in. The admission office checks the record before the patient comes. When he comes the details are checked and confirmed by the admission office.

   5. The patient is admitted and sent to the ward

   6. When the patient has been discharged his name is removed from the waiting list.

   7. Certain checks are made on the waiting list to remove the names of patients who have been admitted through accident and emergency department or who have died to remove their names from the waiting list

7. Statistics
   Come for day case treatment. Special letters are sent to these patients because of preparations needed if general anaesthesia is to be given.

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CHAPTER ELEVEN

FILLING SYSTEM FOR CASE RECORDS AND X-RAY FILMS

INTRODUCTION

This chapter describes filing systems for case records and X-Ray films.

LEARNING OBJECTIVES

The students should be able to-

1. Define filing
2. Describe the three main filing systems.
3. Explain how to convert sequential filing to terminal digit filing system
4. Describe filing equipment used to file case records and x-ray films.
5. Describe the ancillary equipment.

Filing is a systematic way of arranging documents to enable those documents filed to be maintained in good order. The library from which notes are not readily available is a threat to the patients' treatment.

Filing methods

There are three main filing methods used in an organized health records department:-

(i) Alphabetical
(ii) Chronological
(iii) Numerical.

(i) Alphabetical filing system

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Alphabetical filing really has little place in any discussion of the filing of case notes. The main disadvantage of using this method of filing is that it grows very unevenly and spreading of notes or x-ray film can be very difficult if the file is used for many years and the number of files becomes very large.

(ii) Chronological filing system

This is a method of filing where records are filed using dates when the record was created. It cannot be used to file case records except that it can be used to arrange the records inside the case folder. It is not recommended for filing in a big library.

(iii) Numerical filing system

In this system two ways of filing can be adapted:-

(a) Straight numerical
(b) Terminal digit filing system

(a) Straight numerical filing system

This is probably the filing system that comes automatically into most people's minds:- 1 2 3 4 5 6, 1 2 3 4 5 7. It is probably the most suitable method of filing for a small records library where there is no necessity to go into the fairly elaborate detail needed to install a terminal digit system.

(b) Terminal digit filing system

This filing system was first used in united states hospitals and has been the standard method of filing in that country. Anyone starting a new records department would be well advised to start of with it from the beginning. The main difficulties experienced with the traditional sequential filing system are:-

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(i) Grown is at one end of the file because this is the busiest section of the library

(ii) Gaps are usually left after weeding of notes.

(iii) Transposition of figures

(iv) The library is divided into 100 major sections numbering from 00-99. Each major section is again divided into 100 sub-section. This means that the library is now divided into 10,000 sections. Each section should be labeled properly.

(v) The hospital number should be thought of as three pairs of digits e.g. 1 2 3 4 5 6 as 12-34-56. 56 is the terminal digit, 34 is the middle digit, 12 is the primary or first digit.

(a) The record will be filed in the major section appropriate to the terminal digit (last two numbers in section 55)

(b) Within major section 56, it goes behind the sub-section guide appropriate to middle digit (middle two numbers) 34

(c) It is then filed in the order of primary digit 12.

(c) Advantages of terminal digit filing system are:-

(i) New and old records are evenly distribute throughout the records library

(ii) Chance ensures that an equal number of records and loose filing returns each day to each major section. Trials have shown that where 1,500 records are filed

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daily, 150 records will be returned to each section daily.

(iii) There is no annual shift-back, or closing up of notes after weeding, to make room for new records.

(iv) Sorting of notes is simpler.

(v) New staff find the system very much easier to learn than sequential filing- probably because the library is much more static and the 00s are always in the same place.

(vi) Fewer misfiles occur. This is because the filing clerk is concentrating on only two digits at a time

(vii) Tracer cards can be written in advance when preparing a clinic.

(d) Conversion of straight numerical filing to terminal digit order

Before one changes from straight numerical filing system to terminal digit filing system, one should make sure that there is enough:-

Space
Manpower- trained
Shelves constructed
Medical forms
Pre-printed folders

One day should be set on which to start the system. The first patient who comes should be issued with unit number

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01 and the second patient who comes next 02. This should continue until the number 999999 is reached. When an old patient who had been issued with a number in the previous year's comes his record should be pulled out and brought forward to the new unit number. If a patient does not turn up his record should be left in the old file

(e) Filing equipment used to file case records and x-rays

(i) Shelf filing

This I the most used filing equipment and probably more suitable for filing large quantities of notes or x-ray films than anything else. Metal is more suitable than wood. There are some advantage's and disadvantages of shelf filing.

(i) Advantages

(a) Records can easily be filed and pulled quickly since the shelves are open.

(c) More records can be filed on the shelves that in the cabinets.

(ii) Disadvantages

(a) If shelves are constructed high light cannot penetrate the lowers shelves

(b) If shelves are constructed high the shortest clerk has to climb the ladders or the kit stools

(c) Shelves are not dust-proof

(d) Shelves are not fire-proof

(e) Shelves are not water-proof

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Filing cabinets

Filing cabinets are recommended for smaller libraries. They provide ideal filing conditions:-

(a) Good access
(b) Dust-proof
(c) Convenient height
(d) Provide attractive appearance
(e) security

Disadvantages

1. they take up more space
2. More expensive than the shelves
3. Less records are filled (small library)

Suspended filing

This form of filing is never suitable for a large number of case records and x-ray films on the ground of cost and amount of space taken up. Basically it consists of manila pockets hanging from two metal bars and providing a v-shaped space into which notes can be filed. The metal hanging bars have flat tops to indicate the unit numbers filed in each pocket. Suspended filing can be installed in filing cabinet, with addition of special framework on which to hang the pockets or it can take place of the shelves in horizontal units.

Disadvantages

1. Provides less filing space
2. It is expensive to install

Advantages

It is ideal for filling administrative records where files are slim.

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Mobile raking

This is the most economical filing method as far as space is concerned. A series of shelves, contained in a rigid frame with backing, run on rails. One set of shelving is fixed and then there may be three mobile sets- see diagram.

It will be immediately obvious from this that if all five sets of racking were static, far more room would be taken up by the gangways necessary to gain access to both sides of the racks.

Disadvantage

1. No gangways which could cause hold ups.

2. Access may not be possible by more than two clerks at a time.

3. The pushing of the racks could be a formidable task.

4. It is expensive.

Advantage

It saves space.

Circular, carousel filing

These units consist of a series of shelves or suspended files rotating round a fixed spindle- a large version of the carousel described in the section dealing with the master index. Their great virtue is that the files can be placed against a wall and rotated to bring the files that are needed to the front- they are therefore space saving. This type of installation is undoubtedly more expensive that many others. It should be looked at last for interest and for the space saving principle involved.

Elevator files.

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These are very large relatives of the elevator files described in the master index section. They are extremely expensive and quite out of the financial range of most records departments. However, they should be recognized as possible form of filing.
The records are stored in boxes, on trays, as in the smaller index card elevator files. The difference is that elevator files for notes probably extend upwards for twenty feet or ore. As the operator presses the correct button, the shelves will rotate until the correct one presents itself at filing lever. The argument for their installation apart from the convenience of never having to walk, bend or stretch to pull or file records is that valuable ground floor space no longer needs to be allotted to the records library as most of the notes are stored on one or two floors above while still being available at the most convenient point. It is not possible to use this method in a busy library.

There are two other filing methods which must be referred to, but purely on a historical basis because they are unlikely ever to be used in modern records department. They are-

Bound volumes

These have been referred to already in the description of the unit system. As items of historical interest, and sometimes packed into cardboard boxes and filed on shelves. It is hardly necessary to point out that access to a set of notes that is packed in one cardboard box at the bottom of a pile of several such boxes is not easy to get at.

Two other major items of filing equipment should be mentioned.

Dividers

Documents as heavy and at the same time as flexible as case folders and x-ray films need plenty of support. This is provided by dividers for any sort of shelf filing. Ideally these dividers are metal, and reach from the bottom of one shelf to the top surface of the shelf below, being firmly

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attached to both shelves. They thus provide support not only for the files but also for the actual shelf unity by adding rigidity. The metal dividers can be supplied with a rolled edge similar to that used for the shelf units. Notes should be divided every 12" and x-ray films divided after 6" otherwise they soon "drop" and become permanently mis-shaped.

Colour coding

Colour coding can be applied to both notes for a variety of purpose. It is traditional that the ten colours should be used for the ten main divisions of terminal digit filing. Thus each number 1-9 has a different colour. Colour can be used to identify a particular number except that a few individuals suffer from colour blindness. Colour coding can also be used to indicate the year when the record was created. This can be affected by use of coloured cellotape or coloured adhesive levels.

Ancillary equipment

The efficiency of a well laid out. Carefully filed records or x-ray records library can be significantly increased if certain basic items are provided in addition to the most suitable type of shelving.

(a) Filing trolleys

If clerks are to walk round the filing area with armfuls of notes, putting them down each time they have to withdraw or file a record they will be very tired by the end of the day. Trolleys should be strong and large enough to be able to carry heavy loads of case records and x-ray films

(b) Kik stools/ladders

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These are necessary for the shortest clerk to climb on to reach of shelf as high as 7 feet. Any ladder or kid-stool should be light and easy moved.

(c) Sorting equipment

Sorting equipment for case records and x-ray films will certainly be some form of pigeon hole. It is usually very important that this vital task is not attempted with inadequate equipment.

Where terminal digit order is used, it is desirable to have one hundred pigeon holes, one for each terminal digit. These should be clearly numbered. As the files come back to the library from clinics, secretaries, wards, they can be filed straight into appropriate pigeon hole of the sorting unit. The filing clerk responsible for each section of the library will then take on pigeon hole of numbers, sort them out and file them.

(d) Preparation tables.

These tables can be used for sorting the notes for filing for preparing the clinics. There should be enough space for each clerk and the table should be large enough.

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CHAPTER TWELVE

TRACING SYSTEMS

INTRODUCTION

This chapter describes the movement and the whereabouts of all the health records form other departments or other sections in the hospital or even to other hospitals.

LEARNING OBJECTIVES
The students should be able to:-

1. Define tracing.
2. Describe different types of tracing health records.
3. Explain the advantages and disadvantages of each tracing system.

There are three systems of tracing most commonly used in a records library.

(a) The common tracer card

Enough supply of tracer cards is kept in the library. When a record is removed from the filing area it is replaced by the tracer card. When the card is returned to the file the tracer card is removed the notes filed back. The tracer card is

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cancelled and re-used for another set of records – hence the adjective "common". The common tracer card should be made of strong card as it may be used up to 80 times before it is full. The information to be recorded on the tracer card should be as follows:-

Date
Destination (ward or clinic)
Hospital number
Patient's full name
Reason for extraction
Borrower's signature

Some special mention should be made about the date. Where the notes are being extracted for an out-patient clinic, the date recorded on the tracer card should be the date of the clinic not the date of extraction. When the patient is being admitted the date of admission should be included on the tracer card. If the notes are for research or for a letter to be written, the date should be the date of extraction. The tracer card should be one-quarter inch larger than the notes so that it can guide the filing clerk of where the notes should be inserted.

Advantages of the common tracer card

1. It can be used 80 times for different patients.
2. A clerk can prepare a whole clinic in advance while seated.

If tracer cards are used for x-ray envelops size 15" * 18" the tracer card should be more than the envelope so that it can act as a guide to locating the correct place and filing the films back. The same information will be recorded as on the case for record tracer cards.

(b) Library tracer system

Each case record has a small pocket inside the cover in which is held a small card, with the patient's name and the hospital number at the top, and a series of lines below in which borrower and date may be entered, very much on the

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principle still being used by public libraries. When the notes are withdrawn form file the card is marked with date and name of the borrower and filed in a small tracer index. When notes are returned to file, the card is extracted from this index, the entry cancelled and the card filed into the pocket in the folder which is it then refilled.

Disadvantages

1. This system is recommended only for a small library.
2. There is no object left in place of the file to show where the record is.
3. Misfiling is one likely to occur.

Advantage

Because the card is created permanently when the folder is created, the facts are always right.

(c) Personal tracer card

This card is created the same time as the case folder and identified with the patient's name and hospital number. It is filed inside the folder and is only taken out of file when the notes are taken out of the file. The same details as on the common tracer card are entered onto it.

Advantages

1. When the record cannot be traced the details of the previous destination may give accrue as to the destination of the record.
2. No transposition of numbers since the number is created permanently on the card.

The important of the tracer system cannot be overstressed. It is no excuse that notes cannot be found because the tracer is out of date. This will not help the patient. Like any other procedure, the tracer must help to achieve instant availability of the patient's notes when they are needed.

(d) Requisition holder

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This system needs a lot of trust by the one would be borrowers. The borrower writes out a requisition slip for each set of notes required and then sent to records library. The strips are sorted out in order and the notes pulled out. The slips are made of strong document material or thin formal or thin metal. If second borrower makes out a second slip out sends it to the record library where it will be retained in the records pockets until it notes are retained back to the file.

Discoid: -
Very few borrowers would faithfully report it delivery
A register where the borrows can sign must be kept along with the slips

Advantages

(i) Its staff has no tracer cards to use
(ii) The slips forma permanent requisition order

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CHAPTER THIRTEEN

MECHANICAL DOCUMENTATION

INTRODUCTION

This chapter describes how information can be reproduced mechanically once a “master” document has been produced.

LEARNING OBJECTIVES

The student should be able to:-

1. Define mechanical documentation.
2. Explain importance of mechanical documentation.
3. Describe the methods used to carry out mechanical documentation.
4. Describe the information that should be recorded down in carrying out mechanical documentation.
5. Explain how the plates are stored.
6. Explain the advantages and disadvantages of mechanical documentation.
7. Differentiate between the centralized and decentralized systems.

Mechanical documentation is a term used to describe a variety of processes through which information is

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reproduced mechanically once a “master” document containing this information has been created.

IMPROTANCE AND USES OF MECHANICAL DOCUMENTATION

There are various uses of mechanical documentation:-

Reproduction of patient identification details to save the writing by professional, technical and clerical staff of this information. On the hundreds of forms, documents and specimens that are created daily in any hospital and ensuring that these items are correctly and legibly identified.

There are five main types:-

(a) Embossed metal plate
(b) Embossed plastic plate
(c) Foil plate
(d) Stencil
(e) Spirit master

(a) Embossed metal plate

A small metal plate, usually three and a half by one and a half inches is embossed with the patient identification information. The embossing machine has a typewriter – like keyboard which operates a rotation drum containing dies which emboss the characters on the plate.

The plate is fixed in a special holder which is then fed into an imprinter. This imprinter will transfer the information on the plate direct onto documents, or onto self-adhesive labels which may be used to identify forms or specimens.

The plate is taken out of the printing holder and is usually kept in a special small pocket inside the cover of the case folder.

(b) Embossed plastic plate

The principle is similar except, that a plastic plate similar to a back credit card is produced. It can be used in the same way as a metal plate.

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(c) Foil plate

A strip of perforated metal foil is fed into a typewriter, preferably electric, and the information typed on to produce embossed foil similar to the metal plate. This is fixed in a special holder for printing, and can be stored afterwards in the case folder.

(d) Stencil

A strip small stencil in a card frame is fed into a typewriter, again preferably electric, and the patient identification information cut into the stencil in the usual way. This information can then be printed onto forms or labels.

(e) Spirit master

A "master" is prepared by typewriter – this is specially treated paper which has the property of allowing reproduction of the typed information many times. The typed master is then put onto a spirit duplicator and the information transferred to forms or labels.

All these systems must be carefully examined before any judgment can be made on what is most suitable for any particular hospital or department. They vary considerably in price and in quality of reproduction and some are easier for unskilled staff to operate than others. Anyone choosing a system of mechanical documentation will probably wish to bear in mind, as well as the technical advantages of the system, its price, the space it will take up, the noise or other physical aspect of its installation, and the staff available to operate it.

Three general aspects should be considered:-

(a) Information to be recorded.
(b) Storage of plate\stencil\spirit master
(c) Centralized versus decentralized system

(a) Information to be recorded.

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This will vary form one hospital to another and will depend on the use to which the system is to be put. If the aim is solely the identification of all the documents and the specimens. The basic facts to be embossed are:-

Patient's full name
Date of birth
Sex
Hospital number

These will be sufficient to identify the document or specimens without any doubt. But if the system is to be used for addressing purposes, then it is necessary to include the patient's full address and the consultant's name and his address which will consume time.

(b) Storage of plate\stencil\spirit master

One has to weigh both the advantages and disadvantages of retention. Retention will mean that space will be occupied inside the folder. Destruction means that the masters are destroyed each time the documents needed have been produced. Therefore, the masters should be retained.

(c) Centralized versus decentralized system

A decision has to be reached whether all the printing will be done in one office or in the clinics. A study should be carried out in advance to see whether there is enough manpower to carry out this work decently. If the staffs available are not enough then it should be done centrally.

Advantages

1. Accuracy
2. Time saving
3. Uniform (information)
4. Labour saving
5. Legibility
6. Avoids repetitive clerical effort
7. Versatile

Disadvantages

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1. It can be expensive.
2. Some machines take up a lot of space.
3. It can be noisy.
4. May need to train operators.

CHAPTER FOURTEEN

ADMISSION AND DISCHARGE PROCEDURES

INRODUCTION

This chapter describes procedures followed when a patient is being admitted to the ward and when he is being discharged from the ward.

LEARNING OBJECTIVES

The student should be able to:-

1. Describe admission and discharge procedures and their sources.
2. Edit and audit the record.
3. Collect, compile, analyze, disseminate and verify returns.
4. Code and index.

ADMISSION PROCEDURES

Patients should be supplied with a booklet containing information about the hospital before they are admitted. If they are emergency patients then they should be supplied with the booklet after admission. The book should contain the layout of the hospital, visiting times for relatives and

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visitors. The wards should easily be identified. Accommodation for relatives of very ill patients, mothers ill babies should be available. After admission patients should be guided to the ward by a member of staff to the sister or nurse in charge of the ward. If there are ward clerks, then they should be responsible for reception and documentation of patients in the ward. This will ensure a feedback of information to the health records\information department.

SOURCES OF ADMISSIONS

Admissions can come from different sources:-

(i) Accident and emergency department.
(ii) Out-patient clinic
(iii) Other hospitals
(iv) In-patient waiting list.

DISCHARGE PROCEDURES

Admissions and discharge registers should be maintained in all health institutions. The register should provide for full name of the patients, date of birth, admission date, discharge date, diagnosis and the length of stay in the hospital.

EDITING AND AUDITING OF THE HEALTH RECORD

Auditing of the record is the arrangement of all the forms inside the unit folder in a prescribed manner. This can be decided by the hospital concerned. It can either be done chronologically or in order of specialty. This can be done by the ward clerks if they are available.

After the record has been edited it should be returned to the health records department. In the department the records should be crises-checked with the daily bed returns to ensure that all the records have been received in the records department.

After this the records will be coded and then indexed ready for filing back to the library. The health records and information officer should complete all the information that

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is supposed to be contained in it is there. The discharge procedure cannot be complete until the record returns back to the file.

The patient is then given a return appointment to a consultative clinic and this is recorded on his attendance card. When the patient comes back to the clinic his record should be available in the clinic.

THE MEDICAL AUDIT

The medical audit has been defined as "an objective method for applying a yardstick to the quality of professional performance."

It is the method of evaluating the quality of medical care given to a tool to enable the administrator and the medical staff to uncover inefficient service and point away to an improvement of standards in the health facility – a tool of management. It is important to evaluate the care rendered to the patients in terms of lives saved, avoidable deaths, and patients rehabilitated back to society. This evaluation is carried out by the health records committee.

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CHAPTER FIFTEEN

LEGAL ASPECTS OF HEALTH RECORDS AND VARIOUS ACTS RELATED TO THEM

INTRODUCTION

This chapter will describe how to analyze and interpret the legal requirements affecting health records and the various acts related to them. It will concentrate on confidentiality, ownership, security and disclosure of information.

LEARNING OBJECTIVES
The student should be able to:-

1. Analyze and interpret legal aspects concerning:-
    - Confidentiality
    - Disclosure
    - Ownership
    - Retention
    - Security
    - Consent for operations
    - Medical records ethics

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Confidentiality

Information concerning a patient is confidential and should not be released to any unauthorized persons. If a member of the hospital staff improperly discloses any information concerning a patient whereby that patient suffers material loss, the patient can easily sue the hospital and the officer who is in breach of his duty had made any improper disclosure. If a hospital authority is to minimize its risks in the matter, it is suggested that it should have a rule for stick secrecy about all information regarding patients, their diseases, their affairs and the affairs of their families obtained by any officer in the course of this duties. Further it is recommended that:-

(a) No unauthorized information should be given concerning patients or former patients.
(b) Apart from normal replies, enquiries concerning the progress of a patient's illness is to be given except the instruction of the consultant.
(c) Case notes are not produced to unauthorized members of staff.

Disclosure of information

There are five main categories under which contents of patients' records can be disclosed.

1. Consent by the patient which could be expressed or implied.

2. If there is a court order.

3. If the interest of the doctor or hospital cannot otherwise be safeguarded.

4. If transference between hospital, clinics or doctors in the interest of the patient.

5. If there exists a higher duty than the private duty e.g.
   Notification of infectious diseases.
   Notification of births and deaths registration

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Notification of poisons.

(a) Disclosure with patient's consent.
A patient can give his consent for disclosure either expressly or implicitly. Implied consent arises only in certain limited circumstances as, for instance, when records are disclosed to another medical agency for the purpose of continued treatment. Express consent is obtained when the patient signs a document authorizing the hospital to disclose his medical history for some specific purpose.

In general the consent form should always indicate the reason for disclosure, and no disclosure should be made except for that reason. If the reason or purpose changes, specific consent should again be obtained. Where a consent form reaches the hospital, the hospital is at liberty to disclose and the patient would have no ground for complaint if the disclosure was wider than he intended.

In case where requests for clinical information are received from solicitors claiming to be acting on behalf of the patient care should be taken to make sure that solicitors really are acting on behalf of the patient, and not, in fact against him.

Requests from insurance companies and similar bodies should only be acceded to with the patient's written consent but should be referred to the hospital authorities.

(i) To (iii) above represent statutory obligations, whereas (iv) below, are "good causes". A statutory obligation must be complied with, whereas although a good cause should be there, there is no breach of law if it is not.

(iv) claims for sickness benefit under the national insurance acts

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(v) Exchange of records between doctors for research purposes.
(vi) Disclosure to a central body for collective statistical purpose e.g. hospital activity analysis.

In the foregoing instances, it is plain that the disclosure is in the public's interest.

Ownership

The records do not belong to the patients even if the fees has been paid. The records belong to the various health institutions who have created them. In the case of government institutions they belong to the government. Case records of private institutions belong to the institutions because they have contributed to the creation of the records.

Retention

The public records Act stipulates that authorities responsible for public records have a duty to make proper arrangements for selecting those records which should permanently be preserved and for disposal of the rest. There are some records that were spelled out by that act and they should not be destroyed.

(a) Post mortem books
(b) Summaries of clinical notes
(c) Discharge resisters containing diagnosis.

The rest of the health records in the folder may be destroyed. This should be done six years after the patient's last attendance. Each hospital should be able to decide on which records to be destroyed depending on the institution's demands.

Security

It is the responsibility of each and every health institution to ensure that there is security in storage and handling of health records. This security could be maintained by:-

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(a) Providing adequate security in the departmental procedures and use of equipment.
(b) Instructing lay staff in the confidentiality of health records.
(c) Requiring all lay staff to sign a declaration of secrecy
(d) Health records staff accepting responsibility for disclosure of contents of health records in their possession.

Consent for operations

It is a legal requirement that a health institution should obtain consent from patient/client before an operation or anaesthesia is administered to him in order to safeguard it. This only becomes difficult in the case of children and unconscious patients. In the case of unconscious patient, the surgeon should carry own with the necessary procedures. In case of somebody under 16 years it is necessary for a parent or guardian to give consent. In case of emergency consent should not delay the procedure because this could increase the risk. In case the operation is to be done on the child the father is the right person to give the consent but if the father is not accessible, the mother's consent would be acceptable. Failing that of the father, mother then the legal guardian would be obtained.

Married women would give consent on their own right just as single women. It is wise to obtain the husband's consent particularly where sterility may follow an operation. For mentally disordered patients the consent of the nearest relative should be obtained.

Medical records ethics.

In Greece, Hippocrates, known as the "father of Medicine" was born about 460 B.C. He was the first to case out superstition and to practice medicine on scientific principles. He was the author of the Hippocratic Oath, which is pledged by physicians and other health workers including health records personnel. It states in part:

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“whatsoever in my practice or not in my practice I shall see or hear amid the lives of men which ought not to be noised abroad- as to this I will keep silence, holding such things unfitted to be spoken”. That is how the confidentiality of health records originated and should be maintained until today.

CHAPTER SIXTEEN

MANAGEMENT OF SPECIAL HEALTH RECORDS

INTRODUCTION

This chapter discusses the special health records in that the way they are created and the information contained in them and the way they are stored is different from that of general records.

LEARNING OBJECTIVES

The student should be able to: -

1. Define special health records
2. Describe types of special health records.
3. Describe the security and control of special health records.

The special health records are: -

1. Psychiatric records
2. Tuberculosis records (infectious disease records)
3. Radiotherapy records
4. General urinary disease records (STI/STD)

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5. Accidents and deficiently records
6. Maternity records

Special health records are records dealing with special cases needing special health care.

1. Psychiatric records

The way the psychiatric records are maintained is different from the way the general records are maintained. This is prescribed in the mental health Act. The admission of psychiatric patients may be "informal or formal". Formal means a patient is admitted without legal or other formalities and the hospital has no right to detain him in the hospital against his will.

Admissions are carried out in reference to the Mental Health Act. Discharge procedures are also carried out under the same act.

The records officer must ensure that the information inside the record is accurate. The same responsibilities being carried out by the Health Records Officer in a general hospital are the same ones carried out in a psychiatric hospital expect for the addition to the special statutory work arising from the operation of the Mental Health Act

2. Tuberculosis records

   Tuberculosis is one disease (cancer is the other) for which an attempt is made at complete registration to identify that population. Since it is an infectious disease notification must be made to the Medical Officer of Health. A register is maintained for this notification. Copies of the notification of new cases is sent to the chest clinic. A unit record is opened for this patient and the information contained unit should be very comprehensive.

3. Radiotherapy records

   Radiotherapy departments present a special records problem. All radiotherapy cases are supposed to be registered nationally for perpetual follow-up. These

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records could be filed in a separate area but given unit number, and copies of the notes for the same patient from other units in the hospital incorporated in the same unit file. This should be a radiotherapy number given to each patient to be used for national cancer registration. This number is prefixed by the year of first registration and is used in all follow-up correspondence, until the patient dies. The prefix of the year to the registration number is a precaution against confusion with the unit number which will be used for treatment and all other occasions in the hospital.

Security and control of special health records

The special health records should be if possible locked in cabinets and the keys kept by the Health Records/information Officer.

CHAPTER SEVENTEEN

PRIMARY HEALTH CARE RECORDS

INTRODUCTION

This chapter will describe the type of records that are created at the health centres and in the community based health care services.

LEARNING OBJECTIVES

Students should be able to:-

1. Define primary Health Care.

2. Identify the type of services that are rendered at the community.

3. Describe the records created at this level.

Primary Health Care is a basic essential health service to the individual family and the community which is

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accessible, acceptable and affordable by the community and the country with their full participation.

Community health services

These are services rendered to the families and community from birth until death. Health records are to be kept for each individual patient. The role of the community health personnel revolves around this service. The personnel involved in this service are expected to work as a team. The team comprises of doctors in the health contres, health records technicians, physiotherapists, dieticians, counselors, dentists.

Community records

1. Birth registration
   Notification of birth by the hospital still requires the parent to register the birth of the infant within six months. A birth certificate is made out and given to the parent.

2. Ante-natal & post-natal cards
   Expectant mothers who attend the ante-natal clinics are given ante-natal or post-natal cards and they are supposed to bring it to the clinic for every attendance.

3. Health visiting records
   From the birth of the infant health visitors start visiting the infant and they maintain some records. These records will be used later as pre-school health records.

4. School health records
   These records are kept in the schools or in a local clinic or health centre. The information collected is

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very comprehensive including hearing, vision teeth, special test, and type of illness, prophylaxis given, family history and home conditions, parents' occupation education attainments.

5. Treatment cards
   A general card for adults is created when he attends a health centre. This card is filed at the health centre.

6. Charts
   Growth to health charts are created at the child Health Clinics. In these charts immunizations cards are also created. Tally sheets to record the type of conditions treated are recorded in these clinics.

7. Family planning
   Family planning records are created and maintained at the family planning clinics. Parents are also educated on how to plan their families when they attend these clinics.

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CHAPTER EIGHTEEN

ESTABLISHMENT FOR A HEALTH RECORDS DEPARTMENT IN A HEALTH CARE FACILITY

INTRODUCTION

This chapter discusses how to start a new Health Records Department in health care facility where none had existed.

LEARNING OBJECTIVES

Students should be able to:-

1. Carry out a needs assessment
2. Describe the procedures in establishing a health records department
3. Organize various sections
4. Describe commissioning procedures

1. Needs assessment

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A study should be carried out to know what is required in the building of a new department, th staff required, the equipment and the stationery that will be needed. This will require a lot of funds and therefore a lot of caution will be needed in carrying out this study. The only person to do this is the health records/information officer because he is the one conversant with the layout of the department and what to put in the department.

Design and layout

The layout of the department should take care of:-

(a) Working space for all the clerks
(b) Storage space for equipment and files
(c) Lighting and air circulation

The layout of an office can either hinder or promote the work. Consider the amount of work to be done, number of staff to work in it placing the filing clerks where the files are. Ensure enough gangways at least 3 inches apart. Take into consideration telephone points, nail collection, electrical sockets. Weigh out the pros and cons of private or open offices remembering that a supervisor needs an open office to co-ordinate and check the work being done. Look at all these with an open mind from the point of view of staff and the system.

2. Procedures in establishing a health records department.

Before one establishes a health records and information department he should consider two factors:-

(a) The line of command
(b) The number of staff and jobs needed to achieve the desired results.

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The most important function of a health records and information office isto store and produce information when needed by medical and administrative staff. In view of this a list of jobs and staff to carry them out can be made. The line of commandment must be specified. The sections of the records department must be organized to fit one with another:-

(a) The objective should be efficiency, accuracy and speed
(b) The function is to store and provide information
(c) Line of command should be understood and known by all
(d) Staff should have initiative and scope to suggest improvement.
(e) There should be co-ordination of sections and department.
(f) There should be continuity in the system so that work continues in the absence of some staff.
(g) Mobility- for staff to change and learn other jobs.
(h) Incentive- to know why the job is being done to satisfy on completion.

2. Fire precautions

All offices should comply with fire precautions. Fire escapes and extinguishers should be labeled clearly. The health records and information officer should ensure that his staff understand and comply with implications of fie regulations. Lectures on fire drills and precautions should be attended by his staff.

Security of information and property

The library must be secure during office hours and after hours, only authorized persons should have access during the night. It should not be a through way to other offices. Equipment's like typewriters, Dictaphones, kardveyor or machines that are very expensive and the records office should be under lock.

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Proximity of rooms

Rooms with related functions should be near to each other.

Fittings

Telephones, electric socket outlets, hoists between floors, lighting, acoustic filing on walls and ceiling should be fitted properly.

Special sound proofing

If machine like those ones for mechanical documentation are to be used they should be installed on rubber pads.

Storage
Enough storage equipment should be installed if the rooms are to remain tidy, store cupboards should be lockable. Shelves should be at least I inch apart and 7 inches high. Ancillary equipment should also be added in the library.

Space
Detailed analysis of the following must be carried out before space can be allocated.

1. The nature of the function of the records department.

Main functions being registration, appointments, medical secretariat, waiting lists, statistics, bed bureau, microfilming, mechanical documentation, library, storage for notes and x-ray films.

2. General volume of work

This can be estimated by knowing the number of admissions, number for out-patient attendance, number of consulting staff, and any special department.

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3. General operational policies

Is it a seven-day health records service?
What type of secretarial service will be chosen?
How long are the records to be retained?

4. Internal communication.

This is internal communication between the records department and other departments in the institution. The possibilities of physical communication.

(a) Any member can transport any item from one part of the department to another.
(b) There could be a messenger to be sent from one department to another in the institution.
(c) A vertical hoist may be installed between one floor and another and by pressing a button to indicate the floor the hoist will stop
(d) Intercoms installed between sections
(e) Pneumatic tube systems are installed which can hold documents or case folders. This type of installation is rather expensive.

If some hoists are going to be installed it is important that they should be between the records library and the registration area and they are sited at convenient area on the floors that are going to be served.

5. <u>The architect's plan</u>
   The Health Records and Information Officer should know the architect's plan and advice accordingly

6. Detailed planning of sections
   The "nerve centre" of the department is the library and should be allocated enough space. Retention period of the records department should be clearly laid down. The space for shelving must be properly calculated. The type of filing equipment must be known- cabinets, shelf filing. The registration and appointment areas must be easily accessible to all out-patients. It should be attractive to patients/

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clients for comfort and privacy. Open counters is agreeable to the patients.

The secretarial section may be centralized or decentralized depending on the requirements of the institution. The master index may be sited near the registration and appointment areas. An intercom can connect other distant areas of the department.

7. Planning individual rooms

Individual rooms should be planned depending on the functions to be carried out in each unit. Furniture, lighting, telephones and electrical sockets should be fitted in appropriate positions in each room.

The name of the surgeon and the anaethetist should be recorded on this form.

Temperature, pulse, respiration and blood pressure charts.

These may be destroyed after the patient's discharge because they are transitory records unless one is marked for retention by the medical officer of health. Fluid balance and diabetic charts. These once could also be destroyed unless otherwise stated.

Nursing record

Should be included in the unit record.

Notification of discharge and discharge letter

A letter of discharge from a health institution should be given to him and his doctor notified as well. In this letter the drugs prescribed to him should be recorded.

Report forms

The report forms should be gummed onto the mount sheed

Post-mortem reports

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This should be a final document and should be filed in the unit file

Medical social worker's report.

This should also be inserted in the case folders

Pre-testing of medical forms

Before the form is used it may e tested by the users to see whether it has been appropriately designed for the work it is going to be used for.

Reproduction and posting of medical forms

Very often document reproduction is interpreted as 'photocopying'. This is only one method of document reproduction. When a machine is hired photocopying becomes more expensive that when it is bought.

Photocopying

Manufacturers are now supplying new models of photocopying machines at ever increasing rates.

Spirit and wax stencil reproduction

This is very simple yet versatile method of reproduction. The actual medium of reproduction is a sheet of paper with fine clay-like coating.

On the reverse side. This sheet is placed over a sheet of hectographic paper, the clay-like coating on the first sheet actually in contact with the carbon surface of the second sheet. These sheets are typed with the information or handwritten. From the master sheet copies are made

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clamping to revolving drum moistened by a spirit solvent. As the drum is turned, sheets of copy paper are fed through the duplicating machine and dampened carbon impression on the master sheet is reproduced in mirror form on each sheet. From one well-prepared master sheets 300 copies can be produced. Mechanical documentation uses this method also.

Other means of document reproduction

Other means of document reproduction are off-set litho and typeset plates. Off-set litho depends on reproduction by either metal or paper (plate). The plate is placed on the rotary press and copies made. Typeset duplication equipment is the same as most professional printers still use. It is expensive to install.

Before equipment for document reproduction is bought the following points should be considered: -

(a) What kind of documents needs to be reproduced?
(b) Is the material likely to be done single or double-sided?
(c) How many copies will be required?
(d) At what speed are copies required?
(e) What quality of copy is required?
(f) What will be the capital and the running cost – (important to consider)?

Control procedures to medical forms

There should be strict control of all forms and records routine to prevent wistful duplication in inefficient procedures. Obsolete and redundant forms should be removed from circulation and unsatisfactory forms amended where necessary. There should not be any duplication of forms. Forms used for ad hoc investigations should be withdrawn as soon as the investigation is over.

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The records officer should maintain a list of all the forms with a similar function should be kept together. This will give the records officer an opportunity to periodically examine, simplify, or eliminate the forms if necessary.

CHAPTER TWENTY

QUALITY ASSURANCE

INTRODUCTION

This chapter will discuss the quality of care that is supposed to be given to the patient, which may only be possible if proper records are maintained.

LEARNING OBJECTIVES.

Students should be able to:-

1. Define quality assurance.
2. Explain the importance of quality assurance
3. Describe procedures in ensuring quality assurance.

Quality assurance is defined as the customer satisfaction process.

The importance of quality assurance is to be able to:-

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1. Identify cases of patients who may be discharged from a health institution before diagnosed.
2. Identify those patients who have died and why
3. The types and results of health care rendered.
4. The frequency of consultations
5. The occurrences of consultations
6. The occurrences of infections on all cases
7. Any other trends in the work performed.

Procedures in ensuring quality assurance
It is accepted that there are professional and consumer components in quality care. The six (6) dimensions of quality that need to be recognized separately and each requiring different assessment is:-

(i) Access to service
What is our ambulances response to emergencies?
(ii) Relevance to need
Do we adequately provide for the needs of our community as a whole?
(iii) Effectiveness
Do individual patents receive an adequate standard of treatment?
(iv) Equity
Do all members of our community have access to facilities and services?
(v) Social acceptability.
Do we provide for patients' privacy in wards and clinics?
(vi) Efficiency and economy

The assessment of quality is both complicated and multi-dimensional involving the news to individuals and the much under questions of community need. Patients should be allowed to air their views on the services provided to them. How do we come across our patients?

(i) Do we show concern about their feelings and about their need to be kept informed about what is happening to them?

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(ii) Do we provide even the basic amenities to ensure that they are reasonably comfortable when waiting for treatment?
(iii) Do we provide adequate re-assurance about the progress of their treatment?

Procedures in ensuring quality assurance in health records

The health records department may carry out its evaluation of the services rendered to the patients through the health records committee. The committee should see that the health records are complete. The committee should meet regularly and report regularly to the senior medical committee in the health institution.

The functions of the health records committee are:-

1. To recommend to the senior medical committee in the department of health record.
2. To recommend policies for maintains of the health record department.
3. To ensure that there is proper filing, indexing and availability of patient's records.
4. To advise and guide the health records office, medical staff and administration of the institution on the release of information from the records.

In order to evaluate the quality of the record, the medical records officer brings to the notice of the senior medical committee. All records which do not meet the lever of quality. The committee analyzes these cases carefully, and check a sample of those analyzed by the medical records officer as adequate. These will be selected at random from the discharges for the period or specific types of case, varying from month to month so that all types will eventually be analyzed.

The medical audit

This has been discussed fully in the previous chapters. This is "an objectives method for applying a yardstick to

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the quality of professional performance." It is a method of evaluating the quality of medical care given to the patient and it is used as a tool of management to enable the administrator and the medical staff to uncover inefficient service and try to raise the standards of care in health care institutions.

Conclusion

The on-going process of care delivery must not be disturbed by the quality assurance activities such as intensive observations of physician's behavior.

No simple measure of the quality of care given to the patient is available. We have to live with the often unproved casual relationship between process and outcome measures.

Health records officers and clinical epidemiologists acting as health accountants should play an active role in quality assurance programmes without losing their neutrality.

More good health care trials and more effects are needed to improve effectiveness of the existing quality assurance programmes.

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GLOSSARY TERMS

AUDIT

An objective method of applying a yardstick to the quality of professional performance. It is a method of evaluating the quality of medical-care given to the patient.

COMMUNICATIONS

The transmission and reception of messages or any means by which a thought may be conveyed.

CODING

1. Colour coding- giving colours some numbers to identify them e.g. black-0, yellow – 1
2. Disease coding- giving number to international classification of (W.H.O)

CENTRALIZED

Same work being done in one area.

DECENTRALIZED

Opposite of centralized. Same work not being done in one area.

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EDIT
To arrange the forms inside

HEALTH RECORD
Any written information about

INDEXING
Arranging the code numbers in a

MASTER INDEX
An alphabetical key to numerically form

PRE-REGISTRATION
Taking down the identification details of a patient through writing a letter before the patient attends.

TERMINAL DIGIT
Filing of records using the last two digits

UNIT NUMBERS
One hospital number given to one patient throughout his attendance in that particular hospital.

TRACING SYSTEM
Is a method used to find out the whereabouts of all the records at any one given time?

MECHANICAL DOCUMENTATION
This is a term used to describe a variety of processes through which information is reproduced mechanically once a ‘master ‘document containing this information has been created.

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